BASELINE → TREATMENT → FOLLOW-UP

PRRT tests before, between and after four intended administrations

This is the NCCP blood-test sequence with EMA potassium safety and the Irish PRRT service pathway. The post-infusion radiation-assessment scan is different from a response scan. The service confirms bookings, exact dates and any additional follow-up.

Check	Before the course	Before each administration	Between administrations	After the final dose	NET response / safety plan
Blood count / kidneys / liver	Baseline tests	Once 2–4 weeks beforehand AND shortly before infusion	Every 2 weeks	Every 4 weeks for at least 3 months, then every 3 months for the following year	Additional tests or longer follow-up if abnormal
Potassium / renal clearance	Assess renal function and electrolyte risks	Potassium before amino acids; renal function and suitability reviewed	Renal checks during therapy; earlier assessment if ill	Renal monitoring for at least the first year	Fluid and electrolyte plan individualised
Heart / pregnancy / hormones	ECHO or MUGA; pregnancy testing where applicable; hormone plan	Review symptoms and any pregnancy possibility; SSA plan confirmed	Report hormone symptoms, especially during first 48 hours	Follow hormone symptoms and contraception plan	Additional heart tests if clinically indicated
Imaging / clinic	SSTR-positive disease and baseline assessment	Specialist clearance for each treatment	PRRT clinic review; special scan(s) after each infusion for radiation assessment	Response and follow-up imaging as agreed	No universal mandatory tumour-marker or response PET interval
Radiation precautions	Discuss household, travel, work and care needs	Receive written instructions for this administration	Observe each restriction until its own end date	Keep discharge information and tell other treating teams	Medical physicist confirms personal restrictions

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00642 v2 · current Irish Lutathera regimen · EMA Lutathera product information and patient leaflet · updated 14 August 2026 · HSE / Irish PRRT service: introductory patient information

THE WHOLE TREATMENT JOURNEY

Lutetium-177 oxodotreotide (Lutathera / PRRT): treatment and checks

Progressive, unresectable or metastatic, well-differentiated G1/G2 somatostatin-receptor-positive gastroenteropancreatic NET under NCCP 00642a.

Four administrations are planned, usually eight weeks apart. Lutathera is infused over about 30–40 minutes, with a four-hour kidney-protective amino-acid infusion and anti-sickness treatment. The visit is much longer than the Lutathera infusion; an overnight stay may be needed.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before the course	Before each administration	Between administrations	After the final dose	NET response / safety plan
Blood count / kidneys / liver NCCP 00642	Baseline tests	Once 2–4 weeks beforehand AND shortly before infusion	Every 2 weeks	Every 4 weeks for at least 3 months, then every 3 months for the following year	Additional tests or longer follow-up if abnormal
Potassium / renal clearance NCCP 00642	Assess renal function and electrolyte risks	Potassium before amino acids; renal function and suitability reviewed	Renal checks during therapy; earlier assessment if ill	Renal monitoring for at least the first year	Fluid and electrolyte plan individualised
Heart / pregnancy / hormones NCCP 00642	ECHO or MUGA; pregnancy testing where applicable; hormone plan	Review symptoms and any pregnancy possibility; SSA plan confirmed	Report hormone symptoms, especially during first 48 hours	Follow hormone symptoms and contraception plan	Additional heart tests if clinically indicated
Imaging / clinic NCCP 00642	SSTR-positive disease and baseline assessment	Specialist clearance for each treatment	PRRT clinic review; special scan(s) after each infusion for radiation assessment	Response and follow-up imaging as agreed	No universal mandatory tumour-marker or response PET interval
Radiation precautions NCCP 00642	Discuss household, travel, work and care needs	Receive written instructions for this administration	Observe each restriction until its own end date	Keep discharge information and tell other treating teams	Medical physicist confirms personal restrictions

What each check is for

1 Four planned PRRT administrations

T1–T4

PRRT carries radiation to somatostatin receptors on NET cells to help control disease that cannot be removed or has spread. A specialist NET and nuclear-medicine team checks receptor imaging, organ function and your priorities before recommending it.

Ask / act: Confirm the current prescription, tests and support medicines.

Sources: NCCP 00642

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.37